

Gastrointestinal Bleeding

(GI Bleeding)

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Bleeding may occur anywhere along the digestive (gastrointestinal or GI) tract, from the mouth to the anus. Blood may be easily seen by the naked eye (overt), or blood may be present in amounts too small to be visible (occult). Occult bleeding is detected only by [testing a stool specimen with special chemicals](#).

Hematemesis is blood that is visible in vomit. Hematemesis indicates the bleeding is coming from the upper GI tract, usually from the esophagus, stomach, or the first part of the small intestine. When blood is vomited, it may be bright red if bleeding is brisk and ongoing. Alternatively, vomited blood may have the appearance of coffee grounds. It results from bleeding that has slowed or stopped, and the blood looks like coffee grounds because it has been partially digested by acid in the stomach.

Blood may also be passed from the rectum:

- As black, tarry stools (melena)
- As bright red blood (hematochezia)
- In apparently normal stool if bleeding is less than a few teaspoons per day

Melena is more likely when bleeding comes from the esophagus, stomach, or small intestine. The black color of melena is caused by blood that has been exposed for several hours to stomach acid and enzymes and to bacteria that normally reside in the large intestine. Melena may continue for several days after bleeding has stopped.

Hematochezia is more likely when bleeding comes from the large intestine, although it can be caused by very rapid bleeding from the upper portions of the digestive tract as well.

People who have lost only a small amount of blood may feel well otherwise. However, serious and sudden blood loss may be accompanied by a rapid pulse, low blood pressure, and reduced urine flow. A person may also have cold, clammy hands and

feet. Severe bleeding with extremely low blood pressure ([shock](#)) may reduce the flow of blood to the brain, causing confusion, disorientation, and sleepiness. Slow, chronic blood loss may cause symptoms and signs of low blood count ([anemia](#)), such as weakness, easy fatigue, paleness (pallor), chest pain, and dizziness. People with underlying ischemic heart disease may develop chest pain (angina) or have a heart attack (myocardial infarction) because of decreased blood flow through the heart.

(See also [Overview of Digestive Symptoms](#).)

Causes of Gastrointestinal Bleeding

The causes of GI bleeding are divided into 3 areas:

- Upper GI tract
- Lower GI tract
- Small intestine

(See table [Some Causes and Features of Gastrointestinal Bleeding](#))

The most common causes, especially for lower GI bleeding, are difficult to specify because causes vary with the person's age.

However, in general, the **most common causes of upper GI bleeding** are

- [Ulcers or erosions of the esophagus, stomach, or duodenum](#)
- Enlarged veins in the esophagus ([esophageal varices](#))
- A tear in the lining of the esophagus after vomiting ([Mallory-Weiss syndrome](#))

Enlarged Veins in the Esophagus (Esophageal Varices)



Hide Details

This photo shows enlarged veins in the esophagus (arrows).

Image provided by David M. Martin, MD.

The **most common causes of lower GI bleeding** are

- [Polyps of the large intestine](#)
- [Diverticular disease](#)
- [Hemorrhoids](#)
- Abnormal blood vessels (angiodysplasia, arteriovenous malformations [AVMs])
- [Inflammatory bowel disease](#)
- [Colon cancer](#)

Other causes of lower GI bleeding include a split in the skin of the anus ([anal fissure](#)), [ischemic colitis](#), and large-bowel inflammation resulting from radiation or poor blood supply.

Bleeding from the small intestine is rare but can result from blood vessel abnormalities, tumors, or a [Meckel diverticulum](#).

Abnormal Blood Vessels (Angiodysplasia) in the Intestine



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This photo shows an abnormal blood vessel in the wall of the bowel.

Image provided by David M. Martin, MD.

Bleeding from any cause is more likely, and potentially more severe, in people who have chronic [alcohol-related liver disease](#) or [chronic hepatitis](#), who have [hereditary disorders of blood clotting](#), or who are taking certain medications. Liver disease makes bleeding more likely because a poorly functioning liver produces fewer of the proteins that help blood clot ([blood clotting factors](#)).

Medications that can cause or worsen bleeding include the following:

- Anticoagulants (such as heparin, warfarin, dabigatran, apixaban, rivaroxaban, and edoxaban)
- Those that affect platelet function (such as aspirin and certain other nonsteroidal anti-inflammatory drugs [NSAIDs] and clopidogrel)
- Those that affect mood or mental health (such as selective serotonin reuptake inhibitors [SSRIs])
- Those that affect the stomach's protective barrier against acid (such as NSAIDs)

Evaluation of Gastrointestinal Bleeding

GI bleeding typically requires evaluation by a doctor. The following information can help people decide when a doctor's evaluation is needed and help them know what to expect during the evaluation.

Warning signs

In people with GI bleeding, certain symptoms and characteristics are cause for concern. They include

- [Fainting](#) (syncope)
- Severe or prolonged sweating (diaphoresis)
- Rapid heart rate (over 100 beats per minute in the absence of physical activity)
- Passing in stool or vomiting more than 1 cup (250 milliliters) of blood

When to see a doctor

People who have GI bleeding should see a doctor right away unless the only sign of bleeding is blood on the toilet paper after a bowel movement. If people with such findings have no warning signs and feel otherwise well, a delay of 1 or 2 days is not harmful.

What the doctor does

Doctors first ask questions about the person's symptoms and medical history. Doctors then do a physical examination. What they find during the history and physical examination often suggests a cause of the GI bleeding and the tests that may need to be done (see table [Some Causes and Features of Gastrointestinal Bleeding](#)).

The history is focused on finding out exactly where the bleeding is coming from, how rapid it is, and what is causing it. Doctors need to know how much blood (for instance, a few teaspoons or several clots) is being passed and how often blood is being passed. People with hematemesis are asked whether blood was passed the first time they vomited or only after they vomited a few times with no blood.

Doctors ask people with rectal bleeding whether pure blood was passed; whether it was mixed with stool, pus, or mucus; or whether blood simply coated the stool. People with bloody diarrhea are asked about recent travel or other possible forms of

exposure to other agents that can cause digestive tract illness (for instance, bacterial infections).

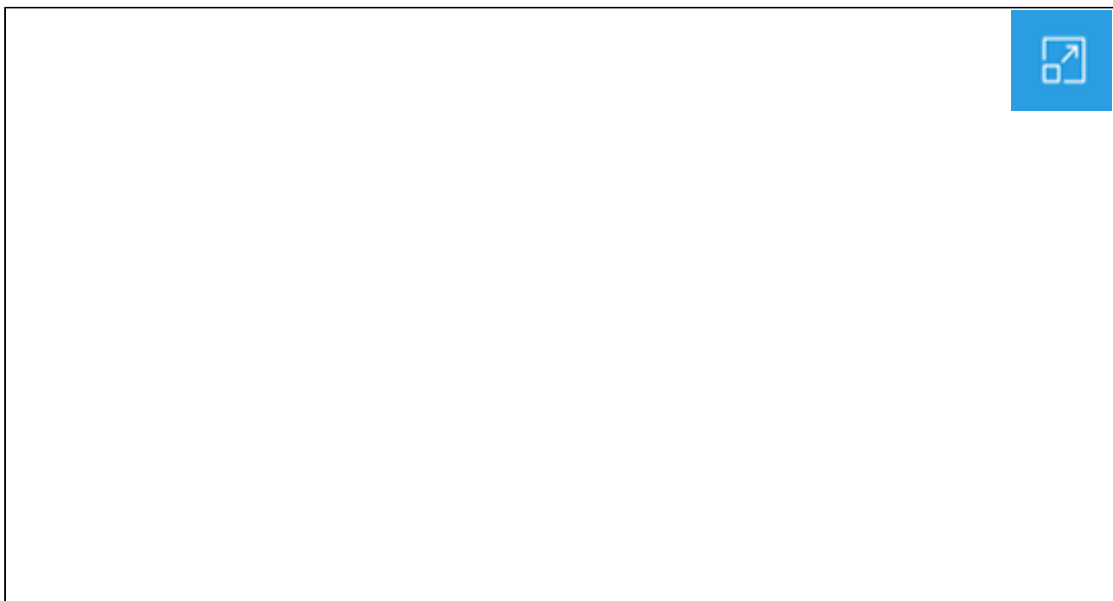
Doctors then ask about symptoms of abdominal discomfort, weight loss, and easy bleeding or bruising and symptoms of anemia (such as weakness, easy exhaustion [fatigability], and dizziness).

Doctors need to know about any current or past digestive tract bleeding and the results of any previous colonoscopy (an examination of the entire large intestine, the rectum, and the anus using a flexible viewing tube). People should tell doctors whether they have [inflammatory bowel disease](#), bleeding tendencies, or liver disease and whether they use any medications that increase the likelihood of bleeding (such as aspirin, NSAIDs, or anticoagulants) or drugs that can cause chronic liver disease (such as alcohol).

The physical examination is focused on the person's vital signs (such as pulse, breathing rate, blood pressure, and temperature) and other indicators of anemia or significant blood loss and shock (rapid heart rate, rapid breathing, pallor, sweating, little urine production, and confusion).

Doctors also look for small purplish red (petechiae) and bruise-like (ecchymoses) spots on the skin, which are signs of bleeding disorders. Doctors also look for signs of chronic liver disease (such as spider angiomas, fluid in the abdominal cavity [[ascites](#)], and red palms) and [portal hypertension](#) (such as an enlarged spleen and dilated abdominal wall veins).

Doctors do a rectal examination to look at stool color and check it for blood and to search for tumors and [fissures](#). Doctors also examine the anus to look for [hemorrhoids](#).



Some Causes and Features of Gastrointestinal Bleeding

Cause*	Common Features†	Tests
Upper digestive tract (indicated by vomiting blood or dark brown, coffee-ground material)		
Ulcers or erosions of the esophagus, stomach, or first part of the small intestine (duodenum)	Pain that • Is steady and mild or moderately severe • Is usually located just below the breastbone • May awaken the person during the night and/or be relieved by eating Painless ulcers can also cause bleeding	Upper GI endoscopy (examination of esophagus, stomach, and duodenum using a flexible viewing tube called an endoscope)
<u>Esophageal varices</u> (enlarged veins in the esophagus)	Usually very heavy bleeding Often in people known to have chronic liver disease such as <u>cirrhosis</u> Sometimes signs of chronic liver disease such as a swollen abdomen and yellowish discoloration of the skin and whites of the eyes (<u>jaundice</u>)	Upper GI endoscopy
<u>Mallory-Weiss tear</u> (a tear in the esophagus caused by vomiting)	In people who vomited one or more times before they started vomiting blood	Upper GI endoscopy



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